

RMAR.0000072295

Bill Of Supply

Name : Mrs. K MEGHANA
Age/Gender : 28 Y F
Contact No : +918332808860
Address : BANGLORE
UHID : RMAR.0000072295

Bill No : RMAR-OCS-777508
Bill/Reg Date : 27.01.2026 16:02
Referred by : Dr. GARIMA JAIN
Center : Cradle Marathahalli
Emp No/Auth Code :

#	Department	Description Of Service	SAC/HSN Code	Qty	Rate	Gross Value	Discount	CGST Rate	CGST Amt	SGST/UTGST Rate	SGST/UTGST Amt	Net Value
1	Lab Tests	GLUCOSE, RANDOM	999316	1	187.00	187.00	18.70	0.00	0.00	0.00	0.00	168.30
2	Lab Tests	THYROID STIMULATING HORMONE (TSH), SERUM	999316	1	517.00	517.00	51.70	0.00	0.00	0.00	0.00	465.30
3	Package Charges	HEMOGRAM (CBP+ ESR)	999312	1	630.00	630.00	0.00	0.00	0.00	0.00	0.00	630.00
4	Round Off			1	0.40	0.40	0.00	0.00	0.00	0.00	0.00	0.40

Payments	Receipt No	Mode	Amount
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Bill Amount: 1,334.40
Total Discount: 70.40

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CIN - U85100TG2009PTC099414
Regd. Office: 1-10-60/62, Ashoka Raghupathi Chambers, 5th Floor, Begumpet, Hyderabad - 500 016, Telangana

GSTIN: 29AADCN1803G1ZP

Address: Opus 143, 1st Cross, 5th Block, Koramangala, Bengaluru, Karnataka

Bill Of Supply

AR.0000072295*

e : Mrs. K MEGHANA
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 Address : BANGLORE
 IID : RMAR.0000072295

Bill No : RMAR-OCS-777507
 Bill/Reg Date : 27.01.2026 16:00
 Referred by : Dr. GARIMA JAIN .
 Center : Cradle Marathahalli
 Emp No/Auth Code :

Department	Description Of Service	SAC/HSN Code	Qty	Rate	Gross Value	Discount	CGST Rate	CGST Amt	SGST/UTGST Rate	SGST/UTGST Amt	Net Value
1 Lab Tests	MATERNAL SCREENING FIRST TRIMESTER-DUAL/DOUBLE MARKER	999316	1	1,600.00	1,600.00	0.00	0.00	0.00	0.00	0.00	1,600.00
2 Lab Tests	NIPT - NON INVASIVE PRENATAL TESTING		1	16,000.00	16,000.00	0.00	0.00	0.00	0.00	0.00	16,000.00

Bill Amount: 17,600.00

Payments	Receipt No	Mode	Amount
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Patient Name : Mrs.K MEGHANA
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UHID/MR No : RMAR.0000072295
Visit ID : RMAROPV778946
Ref Doctor : Dr.Dr GARIMA JAIN

Collected : 27/Jan/2026 04:18PM
Received : 27/Jan/2026 05:36PM
Reported : 27/Jan/2026 05:57PM
Status : Final Report

DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
TSH (Ultrasensitive/4thGen) , SERUM	4.310	µIU/mL	0.270-4.20	ECLIA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies. It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

*** End Of Report ***

Page 1 of 1



Dr.Nisha
M.B.B.S,MD(Pathology)
Consultant Pathologist



SIN No:SPL26003553

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DEPARTMENT OF HAEMATOLOGY

HEMOGRAM (CBP+ ESR)

Test Name	Result	Unit	Bio. Ref. Interval	Method
HEMOGRAM , WHOLE BLOOD EDTA				
HAEMOGLOBIN	11.7	g/dL	12-15	Spectrophotometer
PCV	35.80	%	36-46	Electronic pulse & Calculation
RBC COUNT	4.3	Million/cu.mm	3.8-4.8	Electrical Impedance
MCV	83.2	fL	83-101	Calculated
MCH	27.1	pg	27-32	Calculated
MCHC	32.6	g/dL	31.5-34.5	Calculated
R.D.W	12.2	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	10,250	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	61	%	40-80	Flow cytometry
LYMPHOCYTES	33	%	20-40	Flow cytometry
EOSINOPHILS	2	%	1-6	Flow cytometry
MONOCYTES	4	%	2-10	Flow cytometry
BASOPHILS	0	%	0-2	Flow cytometry
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	6252.5	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	3382.5	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	205	Cells/cu.mm	20-500	Calculated
MONOCYTES	410	Cells/cu.mm	200-1000	Calculated
Neutrophil lymphocyte ratio (NLR)	1.85		0.78- 3.53	Calculated
PLATELET COUNT	344000	cells/cu.mm	150000-410000	Electrical impedance
MPV	8.4	fL	8.1-13.9	Calculated
ESR	09	mm at 1 hour	0-15	Modified Westergren

PERIPHERAL SMEAR

RBC - Predominantly Normocytic normochromic along with Normocytic hypochromic. No nRBCs seen.

WBC - normal in number,morphology and distribution.

Page 1 of 3



Dr.Manjunath T M
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Consultant Pathologist



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DEPARTMENT OF HAEMATOLOGY

HEMOGRAM (CBP+ ESR)

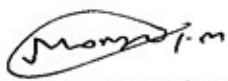
Platelets - normal in number and arranged discretely.

No haemoparasites seen.

Impression - Normocytic normochromic blood picture.

Advice - Correlate clinically.

Page 2 of 3



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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, RANDOM , SODIUM FLUORIDE PLASMA (R)	81	mg/dL	70 - 140	Glucose oxidase

***** End Of Report *****

Result/s to Follow:
THYROID STIMULATING HORMONE (TSH)

Page 3 of 3



Dr.Nisha
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DEPARTMENT OF GENOMICS AND MOLECULAR DIAGNOSTICS

NIPT (NON INVASIVE PRENATAL TESTING) , WHOLE BLOOD IN STRECK TUBE

Pregnancy Type:	Singleton	Gestation Age:	13 weeks 3 days
Fetal Fraction (%):	15.10 %		
Indication:	Abnormal USG findings		
FINAL RESULT:	LOW RISK FOR ANEUPLOIDY		

RESULT	INTERPRETATION
Low risk for Aneuploidy	The probability of specific chromosomal aneuploidy is low
High risk for Aneuploidy	Probability of specific chromosomal aneuploidy is high
No result / Test Failed	Due to indefinite reasons, result was not obtained with the given sample, hence repeat sample testing is advised. Invasive testing is recommended if 'NO RESULT' with repeat sample.

Recommendations:

- The test results should be reviewed and discussed with clinician in the context of patient clinical condition, previous history, pregnancy type, family history for further management.
- Test result should be correlated in conjunction with other diagnostic findings for making final clinical decisions.
- Follow up Invasive testing to be considered further to NIPS high risk test result/ no result and/or in the event of abnormal ultrasound findings.

TABLE: ANALYSED CHROMOSOMES AND OBTAINED RESULT

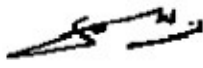
1. COMMON ANEUPLOIDIES				
CHROMOSOMES	RISK	Z -SCORE	TEST RESULTS	REFERENCE INTERVAL
Chromosome 21		-0.35	LOW RISK	-6< Z score<2.8

Page 1 of 8



Dr. THISHYA KALLURI, PhD

Consultant Genomics and Molecular Diagnostics



Dr. SIVA PRASAD SIDDAPURAM, PhD

Head Genomics and Molecular Diagnostics



Dr. SHIBICHAKRAVARTHY KANNAN

M.B.B.S, PhD

Consultant Medical Geneticist

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DEPARTMENT OF GENOMICS AND MOLECULAR DIAGNOSTICS

1. COMMON ANEUPLOIDIES

Chromosome 18		-0.4	LOW RISK	-6< Z score<2.8
Chromosome 13		0.39	LOW RISK	-6< Z score<2.8

2. SEX CHROMOSOMAL ANEUPLOIDIES

CHROMOSOMES	RISK	Z -SCORE	TEST RESULTS	REFERENCE INTERVAL
XO		-0.1	LOW RISK	-3< Z score<3
XXY/XXY		-0.1	LOW RISK	-3< Z score<3
XXX		-0.1	LOW RISK	-3< Z score<3

3. OTHER CHROMOSOMAL ANEUPLOIDIES

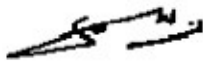
CHROMOSOMES	RISK	Z -SCORE	TEST RESULTS	REFERENCE INTERVAL
Chromosome 1		2.44	LOW RISK	-6< Z score<6
Chromosome 2		-0.41	LOW RISK	-6< Z score<6
Chromosome 3		0.19	LOW RISK	-6< Z score<6
Chromosome 4		0.94	LOW RISK	-6< Z score<6
Chromosome 5		-0.49	LOW RISK	-6< Z score<6
Chromosome 6		-0.52	LOW RISK	-6< Z score<6

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3. OTHER CHROMOSOMAL ANEUPLOIDIES

Chromosome 7		-0.31	LOW RISK	-6< Z score<6
Chromosome 8		-1.15	LOW RISK	-6< Z score<6
Chromosome 9		1.67	LOW RISK	-6< Z score<6
Chromosome 10		-0.14	LOW RISK	-6< Z score<6
Chromosome 11		-0.97	LOW RISK	-6< Z score<6
Chromosome 12		0.4	LOW RISK	-6< Z score<6
Chromosome 14		-0.98	LOW RISK	-6< Z score<6
Chromosome 15		2.38	LOW RISK	-6< Z score<6
Chromosome 16		-0.72	LOW RISK	-6< Z score<6
Chromosome 17		0.91	LOW RISK	-6< Z score<6
Chromosome 19		-0.79	LOW RISK	-6< Z score<6
Chromosome 20		-1.12	LOW RISK	-6< Z score<6
Chromosome 22		-0.94	LOW RISK	-6< Z score<6

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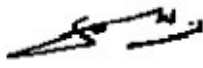
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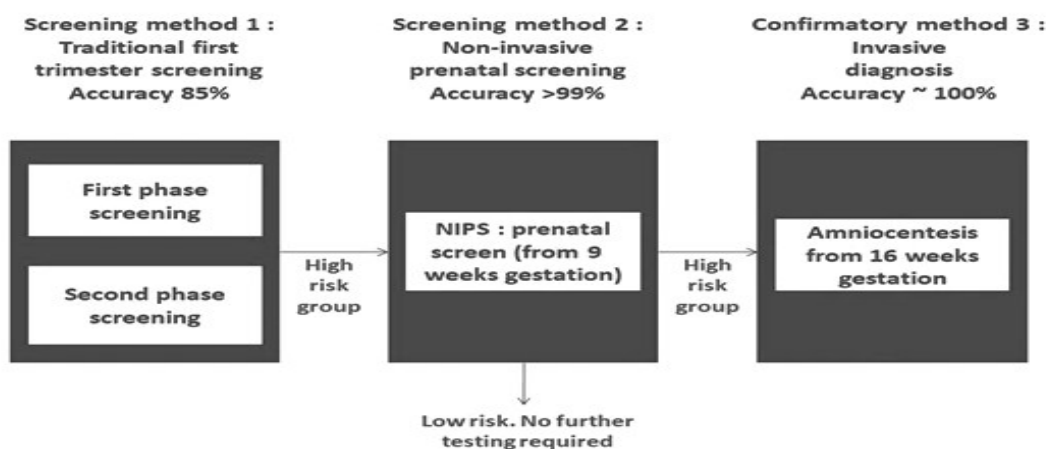
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DEPARTMENT OF GENOMICS AND MOLECULAR DIAGNOSTICS

Clinical Significance / Attributes :

1. NIPS (Non-Invasive Prenatal Screening) test is intended to screen foetal aneuploidies using peripheral blood (cfDNA extracted from peripheral blood) of pregnant women.
2. This test is considered to be a screening test, but not a confirmatory diagnostic test.
3. The method is suitable for both singleton and twin pregnancies, however, the accuracy may be lower in twin pregnancies due to multiple sources of foetal DNA. In twin pregnancy, result can be presented but cannot be differentiated between embryos.
4. The NIPS test can detect aneuploidies of both autosomes and sex chromosomes.
5. This test confers an accuracy of up to 99% in detection of foetal aneuploidy for chromosomes 13, 18 and 21 and slightly lower in other chromosomal aneuploidies. NIPS offers safe and reliable results, reducing the need for invasive tests.
6. The foetal fraction cut-off is 3.5% for the samples with singleton whereas for di-chorionic twin samples it is 7% as per the current methodology.

Prenatal screening pathway



Test Limitations :

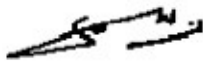
- NIPS is a screening test, and thus all high-risk results should be confirmed using further investigation which may include amniocentesis or Chorionic Villus Sampling (CVS).
- Pregnant women with a high-risk result should be referred for genetic counselling.

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Dr. SHIBICHAKRAVARTHY KANNAN

M.B.B.S, PhD

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SIN No: B85T001545

APOLLO SPECIALTY HOSPITALS PRIVATE LIMITED

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory, Hyderabad

CIN - U85100TG2009PTC099414

Regd. Office: 1-10-60/62, Ashoka Raghupathi Chambers, 5th Floor, Begumpet, Hyderabad - 500 016, Telangana

CAP
ACCREDITED

COLLEGE of AMERICAN PATHOLOGISTS

Address: 101/209 & 210, ITPL Main Road, Kundalahalli, Brookefield, Bangalore, Karnataka



Patient Name : Mrs.K MEGHANA
Age/Gender : 28 Y 2 M 11 D/F
UHID/MR No : RMAR.0000072295
Visit ID : RMAROPV778945
Ref Doctor : Dr.Dr GARIMA JAIN

Collected : 27/Jan/2026 04:18PM
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Status : Final Report

DEPARTMENT OF GENOMICS AND MOLECULAR DIAGNOSTICS

- A negative result does not ensure an unaffected pregnancy, hence recommended to correlate with patient clinical condition, previous history, family history and other laboratory findings.
- Although NIPS is a sensitive test, placental, maternal and foetal mosaicism may lead to false positive results which should carefully be evaluated with other investigations.
- Genetic abnormalities such as micro-deletions, chromosome re-arrangements, translocations, inversions, unbalanced translocations, and uniparental disomy can contribute to false positive results which should carefully be evaluated with other investigations.
- Vanishing twin may contribute cfDNA to the test which can generate paradoxical results, thus few studies suggested to test NIPS after about 8wks from the confirmation of vanishing twin.
- Moderate Z-Scores are associated with lower positive predictive value, hence recommended clinical correlation, and confirmatory test.
- Maternal weight, BMI, early or late gestational age, maternal physical activity may have impact on test sensitivity and test failure.

Test Method :

The peripheral blood sample (8-10mL) was collected in a streck tube from the pregnant women and transported to the lab. Plasma was separated from whole blood, then the cell-free DNA (cfDNA) was extracted from the plasma component. The quantity and quality of cfDNA was confirmed before processing the samples. Each individual sample was labelled with specific barcodes, followed by target sequence amplification. Further, prepared DNA library using amplicon specific DNA of all pooled samples, which was sequenced using Next Generation Sequencing (NGS) technology. The generated data was analysed (**Pipeline Version: 97bef21**) for the determination of chromosomal aneuploidy.

Disclaimer :

Sex determination will not be done at Apollo diagnostics (Apollo health and lifestyles ltd) as per the prenatal diagnostics technique's amendment act 2002, for any prenatal sample.

Reference :

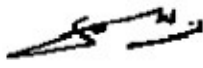
1. Eltabbakh N, Mohasin Y, Jeddy R. Advancements of non-invasive prenatal testing: the role of obstetricians. *Front Med.* 2024;11:1388481.
2. American College of Obstetricians and Gynecologists (ACOG). Current ACOG Guidance: NIPT summary of recommendations. ACOG. 2024.

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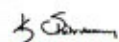
Dr. THISHYA KALLURI, PhD

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DEPARTMENT OF GENOMICS AND MOLECULAR DIAGNOSTICS

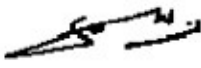
3. Konya M, Czimbalmos A, Loczi L, et al. Genome-wide, non-invasive prenatal testing for rare chromosomal abnormalities: a systematic review and meta-analysis of diagnostic test accuracy. *PLoS One*. 2024;19(11):e0308008.
4. American College of Medical Genetics and Genomics (ACMG). Noninvasive prenatal screening for fetal chromosome abnormalities in a general-risk population: An evidence-based clinical guideline. *ACMG*. 2023.
5. Detection of triploid, molar, and vanishing twin pregnancies by a single-nucleotide polymorphism-based noninvasive prenatal test. Curnow, Kirsten J. et al. *American Journal of Obstetrics & Gynecology*, Volume 212, Issue 1, 79.e1 - 79.e9.

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DEPARTMENT OF BIOCHEMISTRY.

MATERNAL SERUM SCREENING - DOUBLE MARKER (FIRST TRIMESTER) FMF APPROVED

***Method** - Time resolved Immunofluorometry on Delfia Xpress ***Risk ratio** calculated using **LifeCycle 7.0**

MATERNAL DETAILS	RESULT	UNITS
NUMBER OF FETUSES	1	
WEIGHT	91.4	Kg
H/O SMOKING	NON SMOKER	
ETHNIC ORIGIN	ASIAN	
AGE AT TERM	28/08	YEARS/MONTHS
INSULIN DEPENDENT DIABETES	NO	

USG DETAILS		
DATE OF ULTRASOUND	27/01/2026	
GESTATIONAL AGE ON USG	13/3	WEEKS/DAYS
NUCHAL TRANSLUCENCY (NT)	1.4	mm

BIOCHEMICAL RESULTS	RESULT	UNITS	CORR. MOM	METHOD
PAPP-A, SERUM	1755.32	mIU/L	0.76	Immunoflourometry
FREE BETA HCG, SERUM	9.09	ng/mL	0.43	Immunoflourometry

DISORDER (FETUS - 1)	RISK RATIO	RISK CATEGORY	SCREEN RESULT
DOWN'S SYNDROME (BIOCHEMICAL + NT)	1:2982	LOW	SCREEN NEGATIVE
EDWARD'S SYNDROME	1:100000	LOW	SCREEN NEGATIVE
PATAU'S SYNDROME	1:100000	LOW	SCREEN NEGATIVE

Note: The hormonal MOM pattern is not pointing towards risk but sonogical findings show abnormal (hypoplastic) nasal bone. Hence correlation with sonological & clinical findings and NIPT or invasive testing if clinically indicated.

Interpretation:-

- The risks are calculated based on ultrasound gestational age, biochemical results, NT measurement, patient demographics and other risk factors such as IDD.
- Patient specific risks are generated as analytical MoM (Multiples of Median) values, using Fetal Medicine Foundation (FMF) approved assays and software LIFECYCLE version 7.0 from Perkin Elmer.

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MATERNAL SERUM SCREENING - DOUBLE MARKER (FIRST TRIMESTER) FMF APPROVED

3. A screen positive result occurs when the risk for Down's syndrome- exceeds 1:250 and MOM pattern shows free HCG- above 1.8, PAPP A- below 0.4
4. A screen positive result occurs when risk for Trisomy 13/18- exceeds 1:100 and MOM pattern shows free HCG- below 0.5, PAPP A- below 0.4

Limitations:-

1. The results of this test represent only risks and not diagnostic outcomes. Increased risk does not mean that the baby is affected, and further tests must be performed before a firm diagnosis can be made. A low risk does not exclude possibility of Down's syndrome or other abnormalities, as risk assessment does not detect all affected pregnancies.
2. Risk Values are dependent on the information (Patient's demographics, medical history, USG findings) provided by the referring doctor/client/customer. Any change or discrepancy in this information can affect test results and hence it is advisable to ask for repeat risk calculations.
3. Isolated alterations in hormonal values can contribute to statistical risk calculation. Following Maternal factors (adversely affecting risk calculation are gestational hypertension, liver diseases, renal diseases, ovarian tumor, uterine fibroids) and/or Fetal & placental factors (adversely affecting risk calculation are congenital adrenal hyperplasia, IUGR, abdominal wall defects, smith levli opitz syndrome, fetal demise, placental anomalies, retroplacental hemorrhage). These should be considered during case specific interpretation accordingly.

Associated Tests: -

1. NIPT- Non-invasive Prenatal Screening test - from maternal blood for aneuploidy screening.

***** End Of Report *****

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PATIENT REPORT - K MEGHANA

28/01/2026

Requestor: -, -

REQUESTOR:	DOCTOR:	REQUESTOR CODE:	REQUESTOR TYPE:
-	-	-	-

Patient RMAR.0000072295: -, K MEGHANA

PATIENT ID: RMAR.0000072295	LAST NAME: -	FIRST NAME: K MEGHANA	BIRTH DATE: 16/11/1997
ETHNICITY: South Asian	ADDRESS 1: -		

Pregnancy, Calculated EDD: 01/08/2026 (MAEDD: 28.71)

CALCULATED EDD: 01/08/2026	GEST. DATE: 25/10/2025	SELECTED GEST. METHOD: CRL	LMP DATE: 28/10/2025
SMOKING STATUS: Non smoker	INSULIN DEP. DIABETIC: No	NO. OF FETUSES: 1	MONOZYGOUS: No
CHORIONICITY: -	CORRECTED BY CHORIONICITY: -	FERTILIZATION DATE: -	MATERNAL WEIGHT [KG]: 91.4
BIRTH WEIGHT Z-SCORE: -	ASSISTANCE METHOD: -	TRANSFER DATE: -	EGG EXTRACTION DATE: -
EGG DONOR DOB: -	AGE AT EXTRACTION: -	PAST T21 - DOWN'S SYNDROME: -	PAST T18 - EDWARDS' SYNDROME: -
PAST T13 - PATAU'S SYNDROME: -	RISK ASSESSED: At term		

Ultrasound

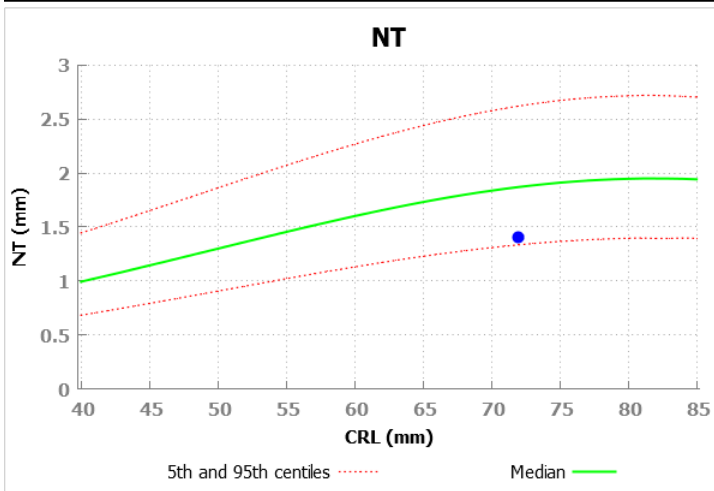
SCAN DATE: 27/01/2026	CRL: 72.5	BPD: -	HC: -
GEST. AT SAMPLE DATE (W + D): 13 w 3 d	CRL (#2): -	BPD (#2): -	HC (#2): -
GEST. AT MANUAL ENTRY (W + D): 0 w 0 d	WEIGHT [KG]: -	AC: -	AC (#2): -

Tests

TEST	SAMPLE ID	DATE	GEST. AT SAMPLE DATE (W + D)	VALUE	UNIT	CORR. MOM	WEIGHT [KG]
hCGb (Signed)	IF00829736	27/01/2026	13 w 3 d	9.09	ng/mL	0.43	91.4
PAPP-A (Signed)	IF00829736	27/01/2026	13 w 3 d	1755.33	mU/L	0.76	91.4
NB (Signed)	-	27/01/2026	13 w 3 d	Absent	-	-	-
NT (Signed)	-	27/01/2026	13 w 3 d	1.4	mm	0.92	-

PATIENT REPORT - K MEGHANA

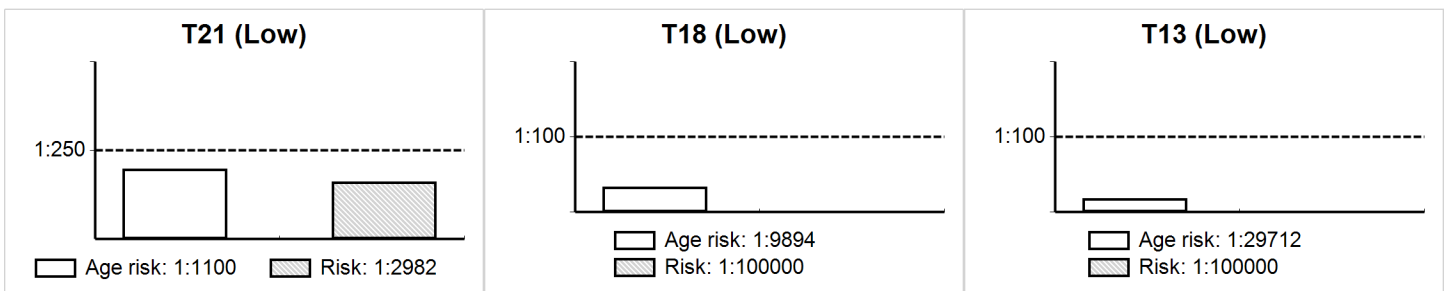
28/01/2026



Distribution median, 5th and 95th centiles obtained from publication: Wright D, Kagan KO, Molina FS, Gazzoni A, Nicolaides KH. A mixture model of nuchal translucency thickness in screening for chromosomal defects. Ultrasound Obstet Gynecol 2008;31:376-83

Risks, Risk assessed: At term

RISK NAME: T21 (Signed)	RISK RESULT: Low	RISK: 1:2982	TWIN RISK RESULT: -	TWIN RISK: -	AGE RISK: 1:1100	CUT-OFF: 1:250
RISK NAME: T18 (Signed)	RISK RESULT: Low	RISK: 1:100000	TWIN RISK RESULT: -	TWIN RISK: -	AGE RISK: 1:9894	CUT-OFF: 1:100
RISK NAME: T13 (Signed)	RISK RESULT: Low	RISK: 1:100000	TWIN RISK RESULT: -	TWIN RISK: -	AGE RISK: 1:29712	CUT-OFF: 1:100



PLEASE NOTE:

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6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only



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